Growing Together

access Care Planning

Writing a PRN Protocol

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Contents

Section	Title	Page Numbers
1	Contents	1
2	Purpose of the guidance	2
3	What is a PRN	3 - 5
4	STOMP Guidance	6
5	LeDeR Guidance	7
6	Completing PRN Protocol Paper	8 - 9
7	PRN Protocol Care Planner	10 - 13
8	Completing PRN Protocol V12/22	14 - 16
8	General Guidance	17
10	Appendices 1) Latin Medication Terms 2) Medication administration Routes 3) Medication Classifications	18 - 20

Purpose of the Guidance

For staff to understand the legislation in relation to the Mental Capacity Act 2005, the Care Act 2014, Health and Social Care Act, and the NICHE Managing Medicines for Adults receiving Social Care in the Community 2017, In relation to the following:

1) Administering Medication

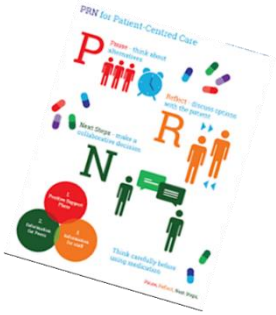


Staff know how to complete a robust PRN protocol, so care staff are able to administer as an when required medications safely and with confidence.

That Staff writing the protocols know where they can get guidance and information from .

That staff can handle and confidently administer medications that they are allocated to do so, with the legislation and policy.

What is a PRN?



PRN is the medical term from the Latin

"*Pro Re Nata*" which means

"*As and When Required*"

Appendix One – Contains a list of Latin Medical Terms in relation to medication.

Appendix Two – Routes for Administration

Appendix Three – Medication Classifications

Please read and review the Appendices

A PRN medication is not a regular medication that needs to be administered all the time.

The medication maybe lead by the following:

1) Service User asking for the medication

2) PRN protocol stating when we need to give the medication to the service user

3) Is it an emergency medication that needs to be administered due to health conditions i.e. epilepsy, or Anaphylactic reaction.

If the medication is not taken it does not need to be recorded as a Refusal, it should be recorded as not required, or PRN not required. As the medication does not need to be administered, it only needs to be given when it is needed.

Examples of PRN Medications

Pain Relief Medication Examples:









Indegestion Medications:







Laxatives / Constipation Medications:







Diarrhoea Relief:







Psychotropic Medication:



The above medications maybe prescribed for short term use, or for when the person needs or requires them. Which can make them a PRN medication.

Please bear in mind that they also be prescribed as a permanent medication, you need to check the medication label and ensure it has the wording which would make it a PRN, if unsure please refer to the dispensing pharmacy or prescribing doctor.

What is the STOMP Guidance?

Stopping over medication of people with a learning disability, autism or both (STOMP)

STOMP stands for stopping over medication of people with a learning disability, autism or both with psychotropic medicines. It is a national project involving many different organisations which are helping to stop the over use of these medicines. STOMP is about helping people to stay well and have a good quality of life.

Psychotropic medicines affect how the brain works and include medicines for psychosis, depression, anxiety, sleep problems and epilepsy. Sometimes they are also given to people because their behaviour is seen as challenging.



People with a learning disability, autism or both are more likely to be given these medicines than other people.

These medicines are right for some people. They can help people stay safe and well. Sometimes there are other ways of helping people so they need less medicine or none at all.

It is not safe to change the dose of these medicines or stop taking them without help from a doctor.

Public Health England says that every day about 30,000 to 35,000 adults with a learning disability are taking psychotropic medicines, when they do not have the health conditions the medicines are for. Children and young people are also prescribed them.

Psychotropic medicines can cause problems if people take them for too long. Or take too high a dose. Or take them for the wrong reason. This can cause side effects like:

- putting on weight
- feeling tired or ‘drugged up’
- serious problems with physical health.

This is why the STOMP guidance exists. To prevent people receiving medication that they don’t need or they are intolerant to when other alternative therapeutic approaches are more appropriate.

What is LeDeR?

Learning Disabilities Mortality Review (LeDeR) Programme

Integrated care systems are responsible for ensuring that LeDeR reviews are completed based on the health and social care received by people with a learning disability and autistic people (aged four years and over) who have died, using the standardised review process. This enables the integrated care systems to identify good practice and what has worked well, as well as where improvements in the provision of care could be made. Local actions are taken to address the issues identified in reviews. Recurrent themes and significant issues are identified and addressed at a more systematic level, regionally and nationally.

A LeDeR review is not a mortality review. It does not restrict itself to the last episode of care before the person's death. Instead, it looks at key episodes of health and social care the person received that may have been relevant to their overall health outcomes. LeDeR reviews take account of any mortality review that may have taken place following a person's death.

LeDeR reviews are not investigations or part of a complaints process, and any serious concerns about the quality of care provided should be raised with the provider of that service directly or with the Care Quality Commission (CQC) via their online system.

Every person with a learning disability whose death is notified to LeDeR will have an initial review of the health and social care they received prior to their death. Using their professional judgement and the evidence available to them, the reviewer will determine where a focused review is required. The person's family has the right to request a focused review. Focused reviews will also be completed for every person from a minority ethnic background.

How LeDeR fits with existing local and national reviews of deaths

There are several different review processes for people who die. For example:

- child death review
- safeguarding adults' review
- review of deaths of people in hospitals

If this is the case, LeDeR will work together to try to avoid unnecessary duplication. Reviewers will make it clear to families where and how the LeDeR process links with other reviews or investigations.

Paper based System Form to Complete

The PRN protocol should cover the following areas:

- Name
- Date of Birth
- Name of Medication and Strength of the Medication
- Dosage of the medication that can be given
- Reason we are giving this medication
- Circumstances we need to give a variable dose of the medication. i.e., if the person complains of anxiety, give 1 tablet, if the person is anxious aggressive behaviour, give 2 tablets. Or using the pain scale 1 – 10 changes the does we may give.
- If using refer to STOMP
- If and when a additional dose can be given, or repeated
- What is the maximum dose that can be given in a 24hour period.

- Additional information i.e., give after food, before food or with food.
- What indicators there are when we would administer the medication
- Actions to take after administering the medication
- What is the medication expected to do?

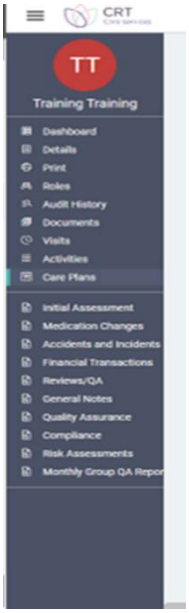
Reading the label and ensuring we have all the relevant information about the medication so we can add this in to the protocol.

Please see on the next page a completed Paper Based Prn Protocol. We may use paper based versions as an interim, so carers know how to administer the medication, whilst waiting for the Care Planner Protocol to be put in place.

PRN PROTOCOL SHEET [PAPERBASED] Example

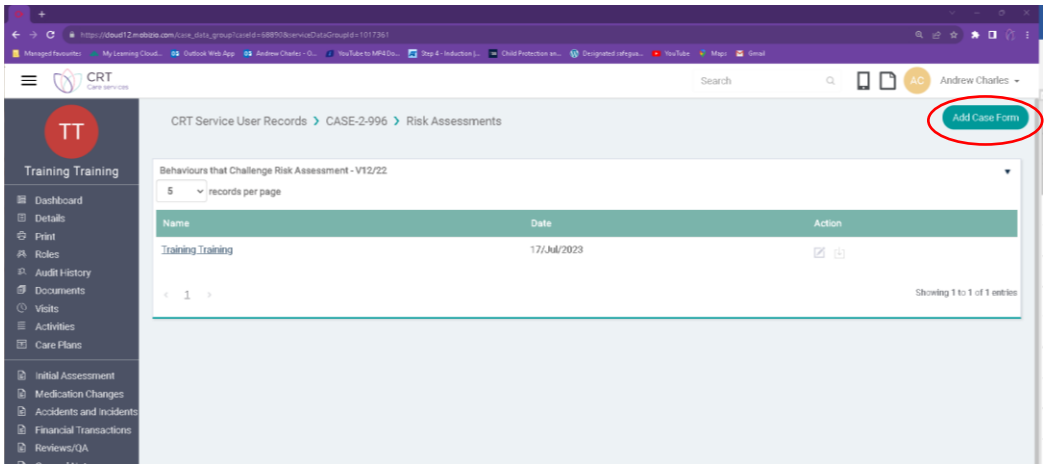
Name	Andy Charles
Date of Birth	27/01/1975
Medicine	1mg Lorazepam
Dose	Take 1 tablet as required for anxiety.
Reason for Medicine	To control the symptoms of anxiety, and help the person return to base line.
Dosage Criteria e.g. Give 1 if or Give 2 if	Give 1mg of the medication, when anxiety attack starts or Andy is showing signs of high anxiety. Such as pacing, incoherent speech, not being able to settle, refusing care and support, wringing hands, and using repetitive speech and phrases.
Reference to pain scale (where appropriate)? State pain scale used	Not pain relief medication
Reference to STOMP plan (where appropriate)	STOMP applies to this medication as it can had sedative effect. It is necessary to give the medication, as person is at risk if medication is not taken, and we can not exceed the max dose.
How often dose can be repeated	1mg every 4 Hours, whilst showing signs of anxiety, as outlined above.
Max In 24 hours	4mg
Further info. e.g. after food	If Andy doe not return to base line after taking up to 4mg, Mental Health Team Crisis Line should be contacted.
Signature	
Date	14/10/2023
Review Date	31/12/2023

Adding a PRN Protocol To Care Planner

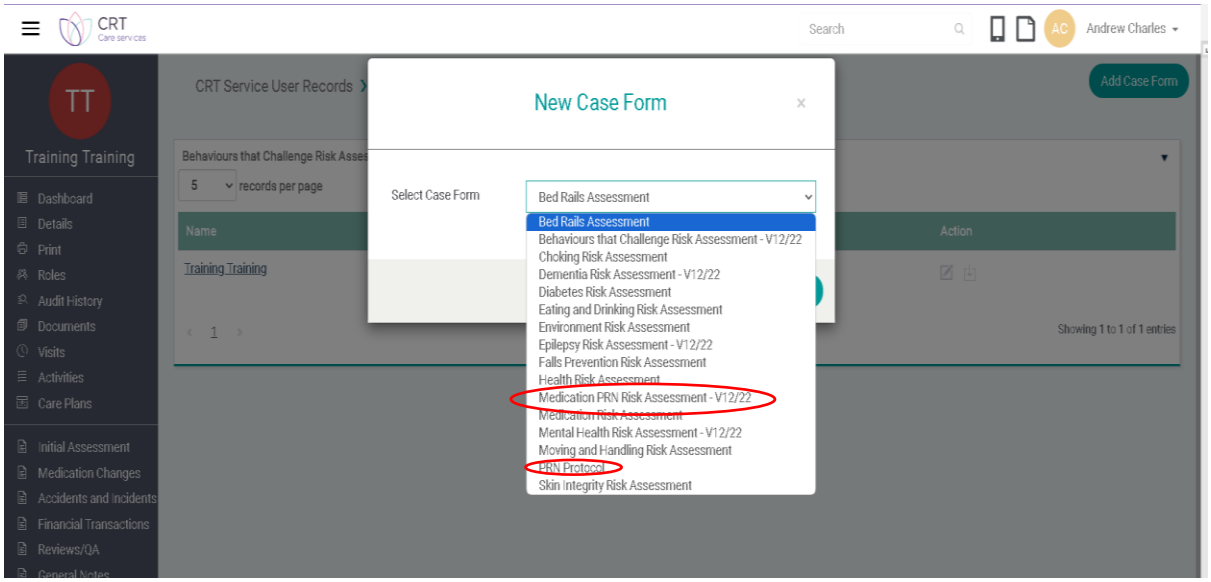


Select the Service User that we need to add the protocol to, and then select the Risk Assessment option.

This will take you to the Screen Below, and you need to select the Add Case Form option circled in red.



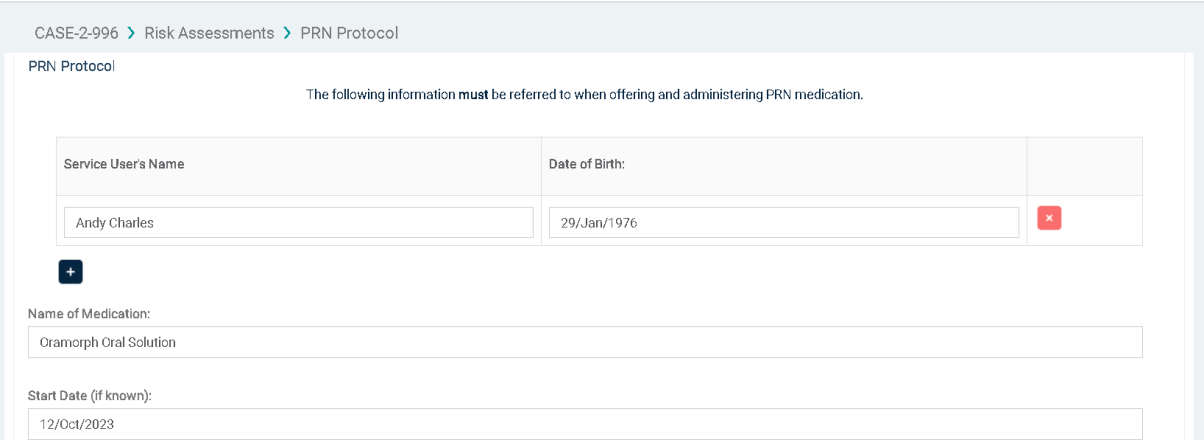
You can now select one of the two PRN forms that the system holds. The next pages will show you how to compile and detail all the information that is needed to make a robust PRN protocol.



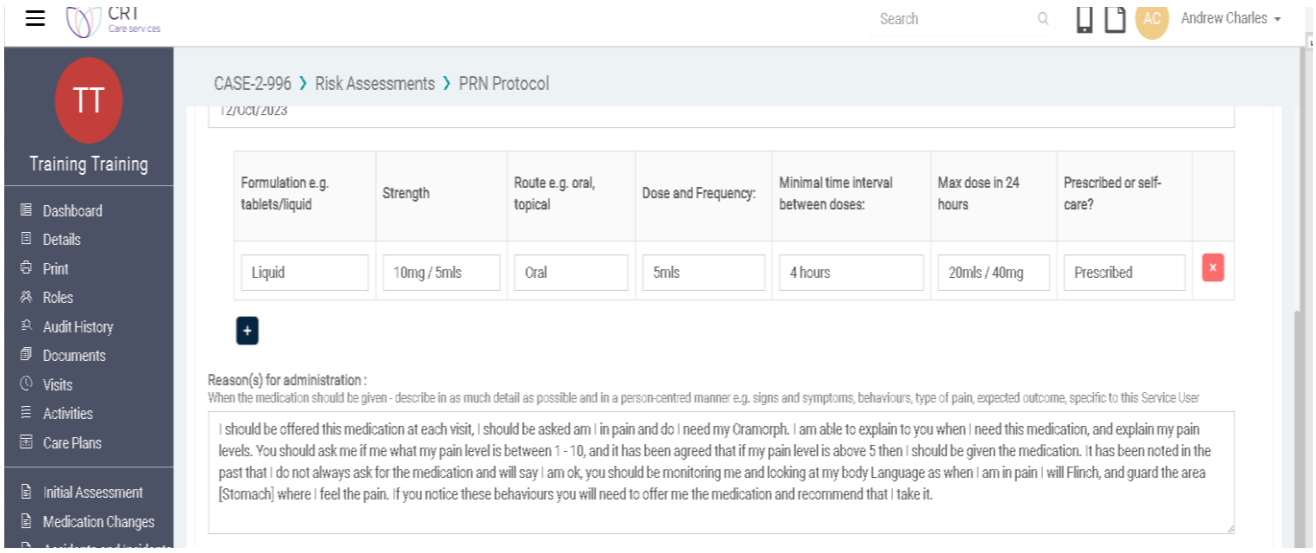
Select which form you wish to complete. This will then load the form, ready for you to complete.

Completing PRN Protocol Form Care Planner

These forms are more robust than the initial paper version, and capture more detail, in them. This in turn makes your Protocol Form more detailed for the carer to understand.



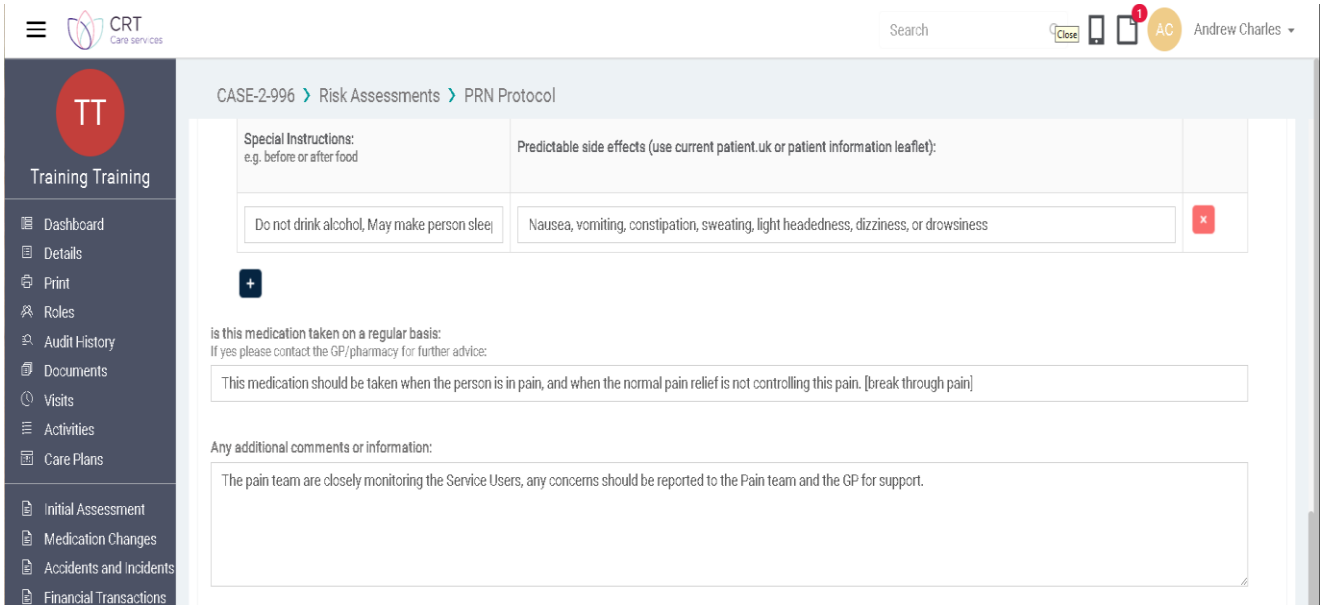
IN this section you enter the Name, Date of Birth, Name of Medicine, and the start date of the medication.





In the above sections we need to detail the Form of the medication Liquid, Capsule, Tablet or Patch. The Strength of the medication, Route that the medication needs to be given, Dose and frequency, Minimal space between doses, Max Dose in 24hrs, and is it a prescribed medicine or Self Care medication.

As can be seen in example above under reasons to administer the medication we need to provide good detailed information so carers know when and how to administer the medication. Remember to include things like body language, phrases, pain scales etc that will help and support carers to make the decision to administer the medication.



In this section we need to complete the detailed information about Special Instructions i.e. do not drink alcohol, may cause drowsiness, may make person sleepy, do not drive, operate machinery etc.

We need to document the side effects of the medication that the carers may witness. We can find information about the side effects from the following places to gain information on the side effects of medication:

1. https://bnf.nice.org.uk/ - You can sign up to your own account, this gives you access to Adult Medications and Child Medications.

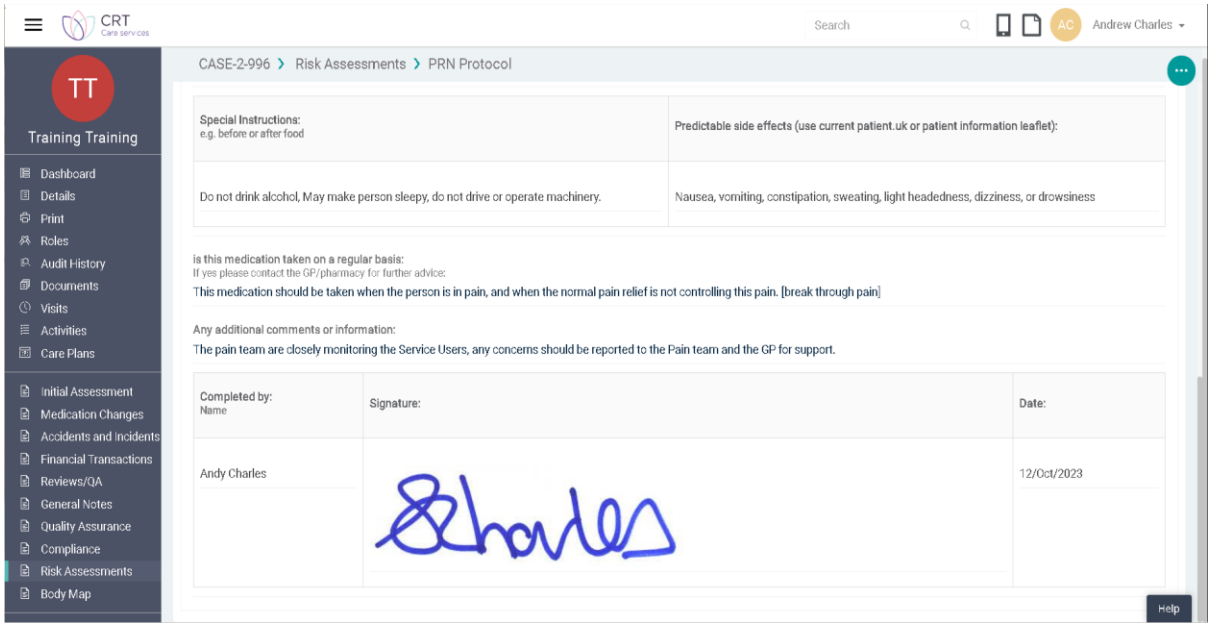
2. https://www.medicines.org.uk - Comprehensive medication database, you can sign up for an account with this site as well.

3. https://www.nhs.uk - Any medicine information that is listed on the NHS site will be acceptable.

Please only use information from the above sites, as other sites such as webmd.com is an American website, and medication guidance is different in America than in the United Kingdom.

Completing the Is this medication taken on a regular basis section – Explain in this section what information that the Pharmacist or the General Practitioner has advised in relation to administering this medication.

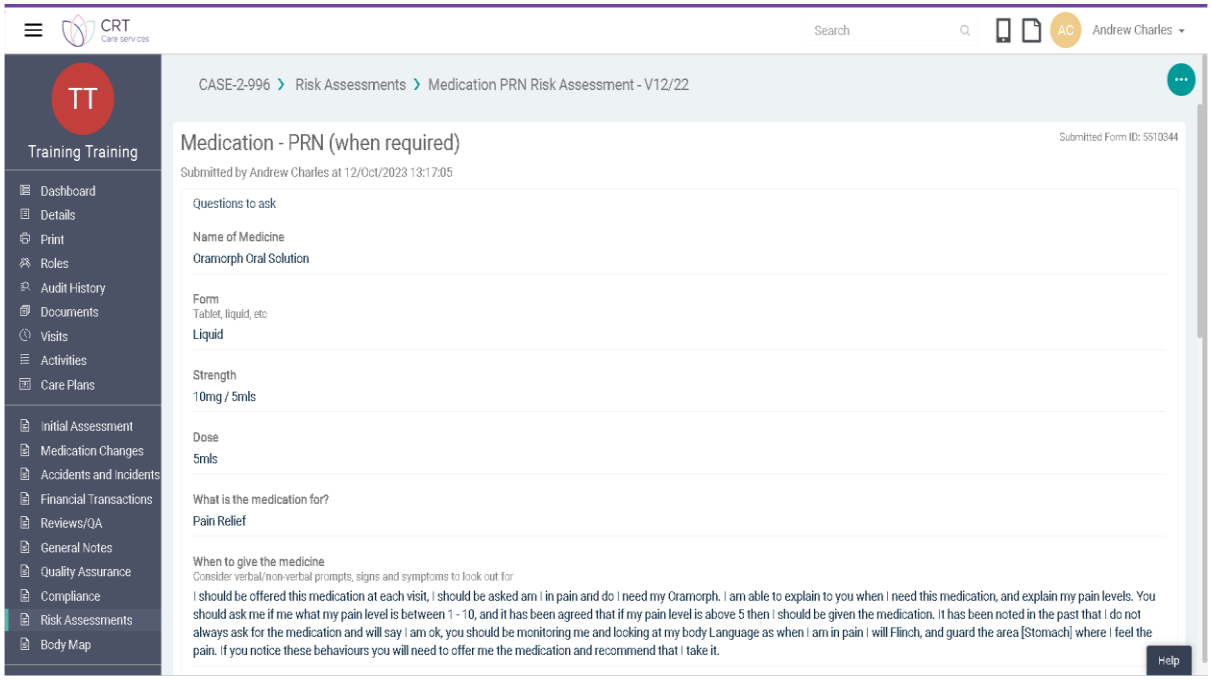
Any Additional comments or information, this is a section that can give the carer more information in relation to medication, and about the medicines that they need to be aware of.



The final Section, you need to enter your name, upload your signature, and enter the date that you completed the protocol.

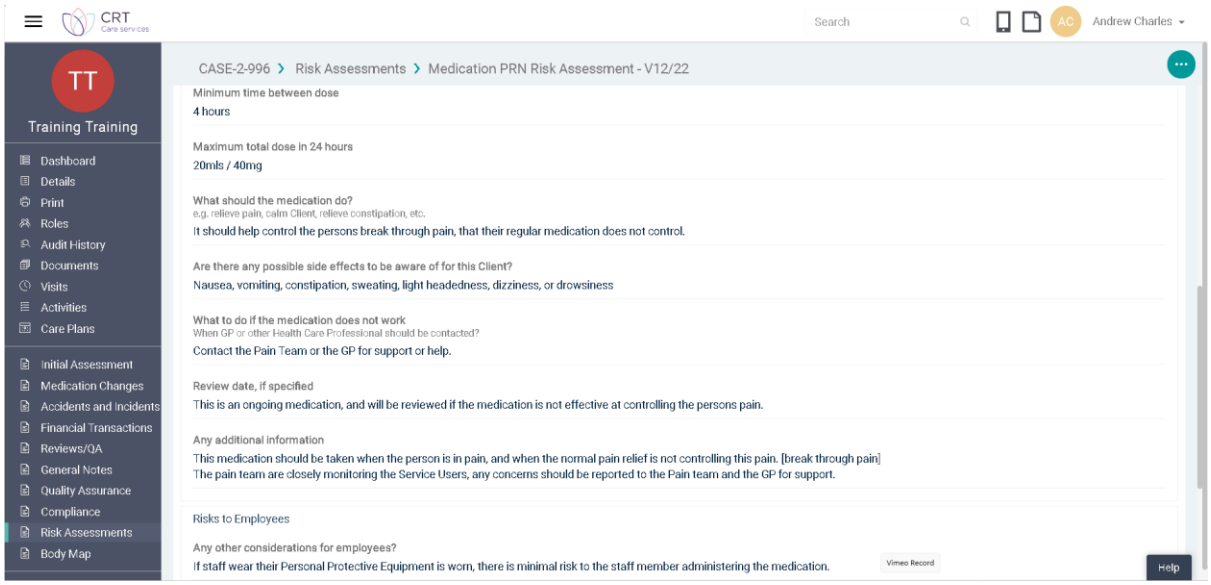
Completing Medication PRN Risk Assessment - V12/22

Form Care Planner





Again we are going to enter the name of the medication, the form of the medication, The Strength of the Medication, the Dosage, What the medication is for, and a detailed explanation of when to give this medication and documenting pain scales, STOMP, prompts and signs and symptoms to look out for, and when to offer the medication.



Enter the minimum time between each dose of the medication.

The maximum dose in a 24hr period. As you can see from the above example, the max dose is explained in mg and in mls, due to this being a liquid medication.

Explain in detail what the medication should do for the individual.

The known side affects of the medication need to be added in to the protocol.

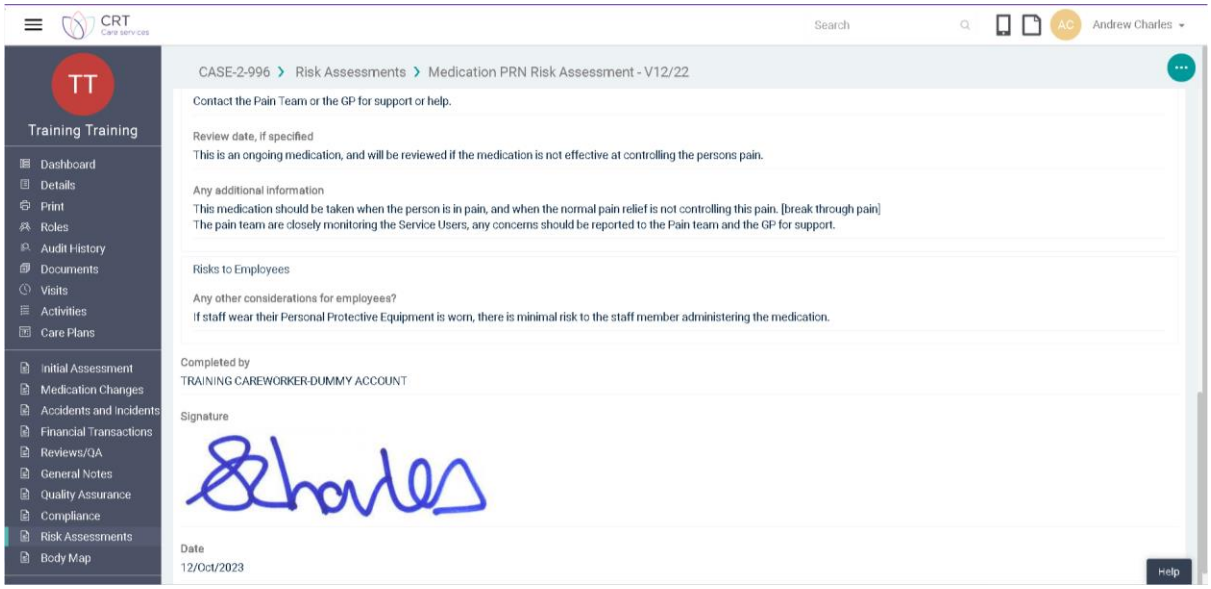
What should the care staff do if the medication is not effective and does not work.

Review date, if it has been specified – Enter any information about when and if this is going to be reviewed, if the person is end of life, it maybe reviewed regularly at check-up appointments for example.

Again enter any additional information that is relevant to the medication and help the carer to make a decision when to administer the medication to the Service User.

This from has a different section to the others which covers what are the risks to staff when administering this medication. Some medications that we administer such as Methotrexarte is Cytotoxic, we would need to add the following section: Women of child bearing age who are being asked to administer cytotoxic medication must be informed of the fact that cytotoxic medication exposure may harm an unborn baby. The importance of always wearing appropriate PPE.

Most importantly we have to reiterate the importance of wearing PPE, when administering medication, as PER medication policy.



Again You need to upload your Signature and date the form you have completed.

General Guidance

When completing the PRN protocol, ensure that you have all the relevant information in relation to the medication.

If you are unsure of frequency of the medication, or how the person should take it, please ring the issuing pharmacy or General Practitioner for advice.

Ensure that you write in to the medication in the care plan section, that staff need to read the PRN protocol before administering the medication. And if there is a variable dose, ensure they write in the notes under the medication the dose they administered to the Service User.

If PRN medication is given regularly or is never given, a referral to the GP should be considered for a review of the service user's medication, as their medical condition may have changed, and the treatment required may need changing. Similarly, if the medication is not having the expected effects, the GP should be contacted.

PRN medication must be added to the Care Plan and Medications, Medication records should clearly state the dosage given and the time it was given. Records should also show if any PRN medication has been offered. This is so that staff visiting later, are aware of what the service user has received and when it was last taken.

It is at the discretion of the service user when they feel their PRN medication is needed; however, administration must remain within the maximum dosage and the required time between doses.

Some PRN medicines are for use in an emergency. Where service users have specific conditions, which may give rise to the need to support with emergency medication, this must be planned in advance, documented within the support plan and individual staff training needs met in advance. For example, Epilepsy Rescue medication.

We also need to bear in mind, mental capacity i.e. can the person consent to taking medication, can they make an informed choice when taking their medication.

If we question the persons capacity in relation to the medication then we will need to carry out a Capacity Assessment and refer to local authority, GP and Pharmacist for support.

Appendix One

Latin Medication Terms

QDS	Quarter Die Sumendus (For Times A Day)
BD	Bis Die (Twice Daily)
OD	Omni Die (Daily)
OM	Omni Mane (Once in the Morning)
ON	Omni Nocte (Once at night)
AC	Ante Cibum (before Food)
PC	Post Cibum (After Food)
PRN	Pro Re Nata (When Required)
QQH	Quarta Quaque Hora (Every Four Hours)
STAT	Immediately
TDS	Ter Die Sumendus (Three Times A Day)

Appendix Two

Medication Administration Routes

ORAL	Via the Mouth
S/C (subcutaneous)	Under the skin
IM (intra muscular)	Into the muscle
IVI (intra venous infusion)	Into the vein
PR (per rectum)	Via the rectum
PV (per vulva)	Via vulva / vagina
Topical	Applied directly to affected area
Transdermal	Absorbed through the skin
Sublingually	Under the tongue
Buccal	In to the buccal cavity
Inhalation	Breathed in to the lungs
Installation	Eye Drops, Ear Drops or Nasal Sprays

Appendix Three

Medication Classifications

Antibiotics	To Treat Infections
analgesics (Pain Killers)	To treat Pain
Antihistamine	To treat allergies
Antacids	Digestive system i.e. indigestion
laxatives	To promote bowel movement
beta blockers	To regulate blood pressure
Anti coagulants	Blood Thinners
Diuretics	To Reduce fluid retention
Psychotropic	Anxiety medications, depression and mood stabiliser.
Cytotoxic Medications	Medications used to treat cancer
Respiratory Medications	Treat breathing conditions such as Asthma, COPD, Bronchitis etc.
Anti-Epileptic Medications	Medications to control Epilepsy or rescue medications that will bring the person out of a seizure.
Hormonal Treatment	Steroid Therapy, Insulin or HRT [Hormone Replacement Therapy]